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C72 - IDENTIFYING SCREENING-LEVEL PREDICTORS OF ENROLLMENT IN A MOBILE MINDFULNESS RCT USING HEADSPACE: A BAYESIAN MODEL-AVERAGED LOGISTIC REGRESSION APPROACH

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Background: Mindfulness meditation is a useful tool with documented effects on various health-related outcomes. Despite the promise of mindfulness meditation, research has identified that some people do not participate in the traditional in-person interventions. Mobile versions, such as those that are app-based, are proposed to solve some of the issues associated with in-person formats, but it remains unclear whether there are differences in enrollers of app-based meditation. Use of app-based meditation is rapidly growing, and identifying if differences exist in enrollment is important to understanding the generalizability of effects and revealing if there are groups being underserved.

Purpose: This study uses screening data from a randomized-waitlist controlled trial testing the app-based mindfulness intervention Headspace to examine predictors of study enrollment.

Method: After excluding five participants with missing responses, 262 participants (age $M = 37.5$ years, $SD = 11.1$; 74.4% non-male) completed a screening survey to assess eligibility and demographic and motivational characteristics. This included the following five factors: age, gender, prior sitting meditation practice in the last six months, and willingness to be randomized into either the meditation or waitlist group. A Bayesian model-averaged logistic regression was performed using JASP to examine predictors of study enrollment. Specifically, a set of 32 candidate models, specified from combinations of the five predictors, were evaluated and averaged.

Results: In total, 140 participants enrolled in the study. The best fitting model included only one predictor, willingness to be randomized to the meditation group. Models including this predictor were supported 16 times more than models excluding it. Participants willing to be randomized to the meditation group were 4.5 times more likely to enroll than those who were not.

Discussion: Even with limited data a motivational characteristic emerged as a predictor of study enrollment. Findings may suggest that providing psychoeducation on the value of being in a mindfulness meditation treatment group is important to increase willingness to enroll. This may be especially important among groups who are less informed on the practice. Examining factors driving willingness and considering whether it is possible to increase willingness of potential participants who may benefit from enrolling in the intervention is an important avenue for future work.

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C62 - INITIATING THE EBATTLE OBESITY PROJECT: USER-CENTERED DESIGN OF A DIGITAL INTERVENTION TO SUPPORT HEALTHY LIFESTYLE AND WEIGHT MANAGEMENT IN ADOLESCENTS WITH OBESITY

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Background: Digital health interventions have the potential to promote healthy lifestyle and behavior change to facilitate weight loss and maintenance in adolescents with obesity. There is however limited knowledge regarding which design features, behavior change techniques (BCTs) and persuasive system design (PSDs) principles that can be most useful.

Aim: In preparation for the eBATTLE obesity trial, the study aimed to explore the needs and wishes of adolescents with obesity in terms of a digital health intervention, and to identify design features, BCTs and PSDs that can enhance healthy lifestyles and behavior change to support weight management.

Method: In total, 15 semi-structured interviews and 6 co-design workshops with key stakeholders explored supportive design features, followed by iterative testing of prototypes, design and development sprints. Key stakeholders ($N = 67$) included in this iterative design and development process included: adolescents with obesity ($n = 23$), next of kin ($n = 9$), healthcare providers ($n = 24$), service designer ($n = 1$) and eHealth experts (e.g., designers, software developers, content developers, scientists) ($n = 10$).

Results: Identified design features, co-creation and user testing with the stakeholders resulted in the development of an evidence-informed app-based intervention for weight management called HabitionTM. Findings show how innovative approaches can be applied to integrate BCTs and PSDs into digital design aiming to support adolescents' needs for health behavior change.

Conclusion: Early and ongoing involvement of adolescents and other key stakeholders is a necessity to develop engaging, motivating, useful and user-centered digital health interventions.

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